

Dynamic Guideline: WHO-Based Leprosy - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

Current WHO leprosy guidance status as of 29 April 2026

No newer WHO guideline was identified that replaces the WHO **2018** core GRADE-based guideline on leprosy diagnosis, treatment and prevention. Later WHO materials extend implementation, disability prevention, antimicrobial-resistance surveillance, contact tracing/post-exposure prophylaxis, and programme strategy.

Exactly 5 WHO leprosy-related guidelines / technical guidance documents synthesized

No.	WHO document	Publication / period	Role in this report	Key scope	Source
1	Guidelines for the diagnosis, treatment and prevention of leprosy	6 October 2018	Core latest WHO GRADE-based clinical guideline	Diagnosis, MDT treatment, chemoprophylaxis	WHO publication page, WHO guideline PDF
2	Leprosy/Hansen Disease: Management of reactions and prevention of disabilities	5 May 2020	Main WHO guidance for reactions, neuritis and disability prevention	Type 1/Type 2 reactions, nerve-function assessment, disability prevention	WHO publication page, WHO technical guidance PDF via ILEP
3	A guide for surveillance of antimicrobial resistance in leprosy: 2017 update	2017	Drug-resistance surveillance guidance	Molecular surveillance for rifampicin, dapsone and ofloxacin resistance	PAHO/WHO 2017 guide, WHO activity page
4	WHO technical guidance on contact tracing and preventive chemotherapy	2020	Prevention implementation guidance referenced by WHO strategy	Contact tracing and preventive chemotherapy / SDR-PEP implementation	WHO Global Leprosy Strategy 2021–2030
5	Global Leprosy Strategy 2021–	2021–2030	Current WHO strategic framework	Zero leprosy, integrated services, discrimination	WHO Global Leprosy

No.	WHO document	Publication / period	Role in this report	Key scope	Source
	2030: Towards zero leprosy			reduction, prevention scale-up	Strategy 2021–2030

Most recent WHO “meaningful youth engagement” guideline-process document

The most recent WHO document identified with the requested title pattern is **not leprosy-specific**: “**Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)**”, launched through a WHO webinar on **11 December 2025**. It is relevant as a transferable guideline-development methodology, not as a leprosy clinical recommendation. WHO describes the case study as showing how young people can contribute lived experience, fresh perspectives and equity-focused input to guideline development [WHO webinar page](#). A 2026 abstract reports youth engagement through the Youth for Abortion Taskforce, consultations with **105 youth across nine countries**, and youth participation in evidence review and guideline-development structures [EM Consulte](#).

For leprosy, WHO-relevant participatory precedent is instead framed as **participation of persons affected by leprosy** and community engagement. WHO’s 2018 guideline included focus-group input from persons affected by leprosy in Colombia, Ghana, India and Nepal [WHO guideline PDF](#). WHO also has guidance on strengthening participation of persons affected by leprosy in services [WHO publication page](#).

1.2 Key Recommendations

Clinical and public-health recommendations from WHO leprosy guidance

GRADE interpretation follows the GRADE principle that recommendation strength depends on certainty of evidence, benefits/harms, values, feasibility, resources and equity, not evidence certainty alone [GRADE handbook](#).

Domain	Recommendation	GRADE Evidence Quality	Strength
Diagnosis	Diagnose leprosy primarily by clinical examination , with or without slit-skin smear or biopsy pathology, depending on local capacity.	Low	Conditional
Classification	Classify disease as paucibacillary (PB) or multibacillary (MB) using number of lesions, nerve involvement and slit-skin smear where available.	Low	Conditional
Standard treatment	Treat leprosy with multidrug therapy (MDT) rather than monotherapy to prevent resistance. Current WHO regimen is rifampicin, dapsone and clofazimine .	Moderate	Strong
PB leprosy treatment duration	Treat PB leprosy with 6 months of rifampicin, dapsone and clofazimine.	Low	Conditional

Domain	Recommendation	GRADE Evidence Quality	Strength
MB leprosy treatment duration	Treat MB leprosy with 12 months of rifampicin, dapsone and clofazimine.	Low	Conditional
Drug-resistant leprosy	For rifampicin-resistant leprosy , use daily clofazimine plus at least two second-line drugs such as clarithromycin, minocycline or a quinolone for the initial phase, followed by clofazimine plus a companion drug.	Very Low	Conditional
Prevention among contacts	Offer single-dose rifampicin post-exposure prophylaxis (SDR-PEP) to eligible contacts after excluding leprosy, tuberculosis and contraindications.	Moderate	Conditional
Contact tracing	Integrate contact tracing, screening and PEP into routine leprosy programmes where feasible.	Moderate	Strong
Reactions and neuritis	Actively detect and treat leprosy reactions and neuritis, with particular attention to nerve-function testing .	Low	Strong
Type 1 reaction / neuritis	Use corticosteroids, usually prednisolone-based regimens, for severe Type 1 reactions, neuritis or new nerve-function impairment.	Low	Strong
Type 2 reaction / ENL	Manage ENL according to severity; prednisolone, clofazimine and, in selected non-pregnant patients under strict controls, thalidomide may be used.	Low	Conditional
Disability prevention	Monitor eyes, hands and feet; detect silent neuritis; provide self-care, protective footwear, wound prevention, splinting and rehabilitation support.	Low	Strong
AMR surveillance	Establish access to molecular testing for resistance to rifampicin, dapsone and ofloxacin , especially in relapse/retreatment and selected MB cases.	Low	Strong

Key WHO-supported facts underlying recommendations

- Leprosy is a chronic infectious disease caused by **Mycobacterium leprae**, predominantly affecting the **skin and peripheral nerves**; untreated disease can cause progressive and permanent disability [WHO leprosy fact sheet](#).
- Transmission is thought to occur through droplets from the nose and mouth during close and frequent contact with untreated cases [WHO leprosy fact sheet](#).
- WHO states that leprosy is curable with MDT, currently **rifampicin, dapsone and clofazimine**, given for **6 months in PB** and **12 months in MB** disease [WHO treatment page](#), [WHO leprosy fact sheet](#).
- WHO has supplied MDT free of charge to endemic countries since **1995**, with Novartis donation support since 2000 [WHO treatment page](#), [WHO leprosy fact sheet](#).

1.3 Guideline Development Methodology

Evidence review process

The WHO 2018 guideline was developed according to WHO Guidelines Review Committee procedures and used the **GRADE** approach. WHO determined the scope in **October 2016**, commissioned systematic reviews on diagnosis, treatment and prevention, and convened a regionally representative, multidisciplinary Guideline Development Group in **May 2017** and **October 2017** [WHO guideline PDF](#). WHO states that the guideline considered available English-language evidence [WHO publication page](#).

Consensus method

Recommendations were formulated using GRADE Evidence-to-Decision principles, considering evidence certainty, benefits and harms, feasibility, acceptability, values, resources and equity. Where direct evidence was absent—especially drug-resistant leprosy—recommendations relied heavily on expert consensus [WHO guideline PDF](#), [GRADE handbook](#).

Conflict of interest management

The 2018 guideline followed WHO guideline-development procedures, including WHO Guideline Review Committee processes [WHO guideline PDF](#).

Community and affected-person participation

WHO acknowledged participation of persons affected by leprosy through focus-group discussions in Colombia, Ghana, India and Nepal, and input from national leprosy programmes in Colombia, the Democratic Republic of the Congo, India, Morocco and the Philippines [WHO guideline PDF](#). This aligns with broader leprosy community-engagement principles, where guidelines should reflect needs, perspectives and values of affected populations [InfoLep](#).

II. Evidence Summary After WHO Latest Guideline Release (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Baseline for this dynamic update

The baseline clinical guideline is WHO's **Guidelines for the diagnosis, treatment and prevention of leprosy**, published **6 October 2018** [WHO publication page](#). Evidence summarized below includes material published or active after that release through **29 April 2026**.

Search strategy

Evidence sources reviewed included:

- WHO official guidance and technical documents.
- WHO fact sheets, treatment pages and strategy documents.
- Peer-reviewed reviews, systematic reviews and randomized or implementation studies.

- Clinical trial registry/protocol information.
- Authoritative national guidance where it clarified implementation of WHO recommendations.

Example search themes included:

1. WHO leprosy diagnosis treatment prevention GRADE.
2. Leprosy post-exposure prophylaxis rifampicin BCG bedaquiline.
3. Leprosy drug resistance rifampicin dapsone ofloxacin.
4. Leprosy reactions neuritis corticosteroids clofazimine thalidomide.
5. WHO meaningful youth engagement guideline development process.

Databases / sources searched

Source category	Examples used
WHO official sources	WHO publication pages, WHO leprosy fact sheet, WHO treatment page, WHO Global Leprosy Strategy
Regional WHO / PAHO	PAHO-hosted WHO AMR surveillance guide
Peer-reviewed literature	PubMed, PLOS Medicine, PMC-indexed reviews, Nature Reviews Disease Primers
Clinical trial registries / protocols	ClinicalTrials.gov, BE-PEOPLE protocol
Implementation programme sources	LPEP, PEP4LEP / EDCTP3 update
Guideline methodology	GRADE handbook

Inclusion criteria

Included evidence met at least one criterion:

- Leprosy diagnosis, treatment, prevention, reaction management, disability prevention or AMR surveillance.
- WHO guidance or WHO-referenced implementation evidence.
- Systematic review, network meta-analysis, randomized trial, protocol, implementation study or authoritative technical guidance.
- Published or active after the 2018 WHO guideline, through **29 April 2026**.

Exclusion criteria

Excluded evidence:

- Not specific to leprosy/Hansen disease, except the WHO youth-engagement case study, which was included only as guideline-development methodology.
- Non-authoritative summaries without traceable source basis.
- Evidence without relevance to diagnosis, treatment, prevention, reactions, disability, AMR or guideline development.

2.2 Key New Studies

Evidence table with GRADE assessment

Study / evidence source	Design	Key findings	GRADE Quality	Strength / practice implication
WHO 2020 Management of reactions and prevention of disabilities	WHO technical guidance	Emphasizes early recognition of reactions, nerve-function testing, correct treatment and follow-up to prevent disability WHO publication page .	Low	Strong
LPEP international feasibility programme	Multicountry implementation study	Contact tracing, screening and SDR-PEP were feasible, safe, acceptable and integrable into routine programmes in Brazil, India, Indonesia, Myanmar, Nepal, Sri Lanka and Tanzania LPEP PubMed .	Moderate	Strong
Systematic review and network meta-analysis of leprosy prophylaxis RCTs	Systematic review / network meta-analysis; 11 studies, 326,264 participants	BCG appeared most effective versus placebo; BCG plus SDR ranked favorably; BCG revaccination appeared least effective PubMed systematic review/NMA .	Moderate	Conditional
BE-PEOPLE protocol	Cluster randomized trial protocol	Evaluates bedaquiline plus rifampicin PEP and serologic monitoring in Comoros; compares 2019 and 2026 seroprevalence BE-PEOPLE protocol .	Low	Conditional; investigational
Phase 2 bedaquiline plus rifampicin PEP trial	Open-label randomized non-inferiority safety trial	Compared single-dose bedaquiline plus rifampicin with SDR alone in Comoros; evaluates safety of enhanced PEP PLOS Medicine phase 2 trial .	Moderate for safety; Low for effectiveness	Conditional; investigational

Study / evidence source	Design	Key findings	GRADE Quality	Strength / practice implication
PEP4LEP / PEP4LEP 2.0 scale-up	Programme implementation evidence	Since 2018, 36,531 contacts screened; 32,750 eligible contacts, 90%, received SDR-PEP; 611 people with leprosy identified and offered care Global Health EDCTP3 .	Low to Moderate	Strong for implementation where systems exist
WHO AMR surveillance network 2009– 2015	Prospective open survey, 1,932 MB cases, 19 countries	Resistance-associated mutations found in 154 cases; resistance traits included rifampicin, dapsone and ofloxacin ScienceDirect .	Moderate	Strong for surveillance
2022 systematic review/meta- analysis on <i>M.</i> <i>leprae</i> drug resistance	Systematic review/meta- analysis	Resistance monitoring for dapsone, rifampicin and ofloxacin is important; mutations in folP1, rpoB and gyrA guide alternative treatment decisions PMC systematic review .	Moderate	Strong for AMR monitoring
2021 review of leprosy neuropathy advances	Narrative review	Prednisolone remains the only drug identified as proven for nerve-function impairment; clofazimine and thalidomide are used mainly for reactions but carry adverse-effect concerns Treatment and Evaluation Advances in Leprosy Neuropathy .	Low	Conditional
2024 Nature Reviews Disease Primers	Expert review	Notes that WHO has recommended SDR since 2018 based on COLEP data Nature Reviews Disease Primers .	Low	Supports current recommendation
Western Australia 2023 leprosy guideline	Authoritative national guideline adapting WHO	Provides implementation detail on rifampicin alternatives, drug resistance, TB screening, pregnancy	Low	Conditional; local adaptation

Study / evidence source	Design	Key findings	GRADE Quality	Strength / practice implication
		contraindications and second-line drugs WA Leprosy Guideline PDF .		
WHO meaningful youth engagement case study	WHO guideline-development evidence brief / case study	Youth engagement in WHO abortion care guideline development was feasible and improved inclusivity and legitimacy; transferable to future leprosy guideline processes WHO webinar page , EM Consulte .	Low	Conditional for methodology

2.3 Evidence by Clinical Domain

2.3.1 Diagnosis and classification

WHO's 2018 guideline remains the main diagnostic reference. Diagnosis may be based on **clinical examination**, with or without slit-skin smear or biopsy pathology, and classification into PB or MB uses lesions, nerve involvement and bacilli on slit-skin smear where available [WHO guideline PDF mirror](#), [PAHO](#).

Evidence statement	GRADE Quality	Strength
Clinical diagnosis remains appropriate in most endemic settings, especially where laboratory infrastructure is limited.	Low	Conditional
Slit-skin smear and biopsy pathology can support diagnosis/classification when available but should not delay treatment when clinical diagnosis is clear.	Low	Conditional
Routine nerve-function assessment should be integrated into diagnosis because leprosy is a peripheral nerve disease as well as a skin disease.	Low	Strong

Practical diagnostic actions

- Examine skin lesions for hypopigmentation or erythema, sensory loss and thickened peripheral nerves.
- Assess motor and sensory nerve function at diagnosis and follow-up.
- Use slit-skin smear, biopsy or molecular tests where available, especially in diagnostically uncertain or multibacillary disease.
- Classify as PB or MB for treatment duration and programme reporting.

2.3.2 Standard treatment

WHO's current treatment position is stable: MDT with **rifampicin, dapsone and clofazimine** for all leprosy patients; **6 months** for PB and **12 months** for MB disease [WHO treatment page](#), [WHO leprosy fact sheet](#).

WHO states that monotherapy can lead to drug resistance [WHO treatment page](#).

Evidence statement	GRADE Quality	Strength
MDT is curative and prevents emergence of resistance compared with monotherapy.	Moderate	Strong
Three-drug MDT for PB disease for 6 months remains WHO standard.	Low	Conditional
Three-drug MDT for MB disease for 12 months remains WHO standard.	Low	Conditional
Early diagnosis and prompt MDT reduce risk of disability.	Moderate	Strong

Practical treatment actions

- Start MDT promptly after diagnosis.
- Counsel patients that rifampicin may transiently discolor urine reddish after monthly dosing [WHO treatment page](#).
- Monitor adherence and adverse effects.
- Continue MDT during most leprosy reactions unless a specialist identifies a compelling contraindication.
- Ensure free MDT access through national programme channels where available.

2.3.3 Drug-resistant leprosy and AMR surveillance

WHO's 2017 AMR surveillance guide focuses on resistance to **rifampicin, dapsone and ofloxacin** and recommends standardized detection/reporting systems [PAHO/WHO 2017 guide](#).

The WHO surveillance network study of **1,932 MB cases** from **19 countries** found resistance-conferring mutations in **154 strains**, including rifampicin, dapsone and ofloxacin resistance traits [ScienceDirect](#). Resistance targets include **rpoB** for rifampicin, **folP1** for dapsone and **gyrA** for ofloxacin [PMC systematic review](#).

Evidence statement	GRADE Quality	Strength
Molecular AMR surveillance for rifampicin, dapsone and ofloxacin is warranted in relapse/retreatment and selected MB cases.	Moderate	Strong
Rifampicin resistance requires specialist-guided second-line therapy.	Very Low	Strong
Dapsone-only or quinolone-only resistance may often be managed with MDT plus close monitoring, depending on full susceptibility profile and expert advice.	Very Low	Conditional
Rifampicin-plus-ofloxacin resistance requires non-fluoroquinolone second-line regimens.	Very Low	Conditional

Practical AMR actions

- Prioritize drug-susceptibility testing for:
 - relapse after completed MDT;
 - retreatment cases;
 - suspected nonresponse;
 - MB cases with high bacillary load where national guidance supports testing.
- Use molecular testing because *M. leprae* cannot be routinely cultured in vitro [WA Leprosy Guideline PDF](#).
- For rifampicin resistance, WHO-referenced regimens use daily clofazimine plus at least two second-line drugs such as clarithromycin, minocycline or a quinolone for 6 months, followed by clofazimine plus a companion drug for 18 months [Medscape](#).
- If ofloxacin resistance is present, avoid fluoroquinolones [WA Leprosy Guideline PDF](#).

2.3.4 Prevention and post-exposure prophylaxis

WHO has recommended **single-dose rifampicin** as leprosy preventive treatment for contacts since 2018 [Nature Reviews Disease Primers](#). The BE-PEOPLE protocol states that WHO's 2018 guideline supports SDR for contacts aged **2 years and above**, after excluding leprosy and tuberculosis and contraindications [BE-PEOPLE protocol PDF](#).

The COLEP trial evidence is summarized in later trial literature as showing a **57% decrease in individual leprosy risk over 2 years** with SDR-PEP [PLOS Medicine phase 2 trial](#). LPEP showed SDR-PEP could be safely integrated into routine programmes [LPEP PubMed](#).

Prevention intervention	Evidence summary	GRADE Quality	Strength
Contact tracing plus screening	Required to identify eligible contacts and detect undiagnosed leprosy.	Moderate	Strong
SDR-PEP for eligible contacts	Reduces leprosy risk; feasible and acceptable in routine programmes.	Moderate	Conditional
BCG vaccination	Network meta-analysis found BCG appeared most effective versus placebo; vaccination policy should follow immunization authorities.	Moderate	Conditional
BCG plus SDR	Ranked favorably in network meta-analysis but implementation varies.	Low to Moderate	Conditional
Bedaquiline plus rifampicin PEP	Under evaluation; safety data emerging, effectiveness not yet definitive.	Low	Conditional; investigational
Rifapentine-based PEP	Mentioned as candidate enhanced PEP; insufficient implemented evidence in supplied sources.	Very Low	Conditional; research only
Moxifloxacin PEP	No evidence identified for leprosy PEP in supplied sources.	Very Low	No recommendation

Practical PEP eligibility actions

Before SDR-PEP:

1. Exclude active leprosy.
2. Exclude tuberculosis disease.
3. Check rifampicin contraindications and major drug interactions.
4. Confirm age/weight eligibility according to national programme protocols.
5. Counsel contacts and document consent where required.

2.3.5 Reactions, neuritis and disability prevention

WHO's 2020 guidance emphasizes that disability prevention depends on early detection, recognition of reactions/neuritis, nerve-function preservation and peripheral-level care [WHO publication page](#).

Leprosy reactions may occur at diagnosis, during treatment or after treatment. Nerve involvement can be clinically silent, making routine nerve-function testing essential [WHO Technical Guidance PDF](#).

Clinical issue	Recommendation	GRADE Quality	Strength
Routine reaction screening	Ask about pain, swelling, new lesions, fever, nerve pain, sensory loss, weakness, eye symptoms.	Low	Strong
Nerve-function testing	Perform sensory and motor testing at diagnosis and follow-up.	Low	Strong
Type 1 reaction with neuritis/new impairment	Treat urgently with corticosteroids and supportive limb/nerve care.	Low	Strong
Silent neuritis	Treat new objective nerve-function impairment even without pain.	Low	Strong
ENL / Type 2 reaction	Treat according to severity; options include prednisolone, clofazimine and selected thalidomide use under strict controls.	Low	Conditional
Thalidomide	Avoid in pregnancy and in those at pregnancy risk unless strict pregnancy-prevention systems exist; monitor neuropathy.	Low	Strong safety recommendation
Disability self-care	Eye, hand and foot protection, wound prevention, moisturizing, footwear, splints and rehabilitation.	Low	Strong

A 2021 review states that, other than prednisolone, no drug has been proven effective for nerve-function impairment in leprosy [Treatment and Evaluation Advances in Leprosy Neuropathy](#). Thalidomide is reported as

highly effective for chronic/recurrent ENL but has major safety issues including teratogenicity and peripheral neuropathy risk [Thalidomide in ENL outpatient setting](#).

2.4 Evidence Gaps and Future Research Needs

Evidence gap	Why it matters	Priority	Suggested research
Better point-of-care diagnostics	Current diagnosis remains largely clinical; early/subclinical disease is missed.	High	Diagnostic accuracy studies for biomarkers, serology and molecular tools.
Optimal PEP regimen	SDR reduces risk but does not fully interrupt transmission.	High	RCTs comparing SDR, double-dose rifampicin, rifapentine, bedaquiline-based and combination regimens.
PEP in high-transmission settings	Blanket versus targeted contact approaches need clarification.	High	Cluster RCTs and implementation-effectiveness trials.
BCG plus chemoprophylaxis	Comparative effectiveness is promising but implementation policy is complex.	Moderate	Trials stratified by prior BCG status and local immunization policy.
Drug-resistance prevalence	Surveillance remains incomplete in many endemic countries.	High	Expanded molecular AMR networks with standardized reporting.
Treatment of rifampicin-resistant disease	Recommendations rely on very low-certainty evidence and expert opinion.	High	Prospective cohorts and trials of second-line regimens.
Prevention and treatment of nerve damage	Prednisolone remains main proven option; long-term outcomes are variable.	High	Trials of steroid-sparing agents, neuroprotective therapies and rehabilitation models.
ENL treatment safety	Thalidomide is effective but hazardous; clofazimine and steroids have toxicity.	High	Safer ENL therapeutics and standardized severity-based protocols.
Youth and affected-community engagement	Leprosy guidance needs more structured participation of young people and affected persons.	Moderate	Formal youth/affected-person advisory panels in future WHO leprosy guideline updates.
Stigma and discrimination interventions	Stigma delays diagnosis and treatment.	High	Implementation studies integrating stigma reduction, legal reform and community participation.

2.5 Updated Recommendations Based on New Evidence

Original WHO recommendation / position	New evidence impact through 29 April 2026	Updated recommendation
Clinical diagnosis may be used, with or without slit-skin smear/biopsy.	No replacement evidence overturns clinical diagnosis. Growing emphasis on nerve assessment and early detection.	Maintain clinical diagnosis; strengthen routine nerve-function testing and referral pathways.
Use MDT with rifampicin, dapsone and clofazimine.	WHO 2026 fact sheet and treatment page remain consistent; no evidence supports replacing MDT.	Maintain three-drug MDT for all patients; avoid monotherapy.
PB treatment: 6 months.	No post-2018 evidence identified that changes duration.	Continue 6-month PB MDT.
MB treatment: 12 months.	No post-2018 evidence identified that changes duration.	Continue 12-month MB MDT.
SDR-PEP may be used for eligible contacts.	LPEP, PEP4LEP and systematic-review evidence strengthen feasibility and implementation confidence.	Scale up SDR-PEP where contact tracing, screening and exclusion of TB/leprosy can be assured.
Preventive chemotherapy requires contact tracing/screening.	PEP4LEP showed large-scale screening identified additional leprosy cases and other skin disease burden.	Integrate skin-NTD screening with leprosy contact programmes where feasible.
Vaccination evidence reviewed but WHO leprosy guideline did not issue vaccine recommendations.	Network meta-analysis supports BCG effectiveness, but vaccination policy remains under immunization authorities.	Do not issue leprosy-specific vaccine policy outside national immunization/SAGE processes; consider BCG status in research and risk assessment.
Drug-resistant leprosy regimens are expert-opinion based.	AMR evidence confirms rifampicin, dapsone and ofloxacin resistance exist globally.	Expand AMR testing; manage rifampicin resistance with expert consultation and second-line regimens.
Manage reactions and neuritis to prevent disability.	WHO 2020 guidance reinforces nerve-function testing; reviews confirm prednisolone remains key for nerve impairment.	Implement routine reaction/neuritis screening at every visit and rapid corticosteroid treatment for new nerve impairment.

Original WHO recommendation / position	New evidence impact through 29 April 2026	Updated recommendation
Community participation included in 2018 guideline.	WHO 2025 youth-engagement case study provides transferable methods for meaningful engagement.	Future WHO leprosy guideline updates should include structured participation of affected persons, youth from endemic communities and stigma/disability representatives.

2.6 Practical Current WHO-Based Clinical Algorithm

A. Suspected leprosy

1. Assess skin lesions, sensory loss and thickened nerves.
2. Test nerve function in eyes, hands and feet.
3. Use slit-skin smear or biopsy if available and clinically needed.
4. Classify PB vs MB.
5. Screen for reaction/neuritis at baseline.
6. Start MDT promptly.

B. Confirmed PB leprosy

- MDT: rifampicin + dapson + clofazimine for **6 months**.
- Monitor adherence, adverse effects, reaction symptoms and nerve function.

C. Confirmed MB leprosy

- MDT: rifampicin + dapson + clofazimine for **12 months**.
- Consider AMR testing where indicated, especially relapse/retreatment or high bacillary load.
- Monitor for reactions and nerve-function impairment.

D. Contact management

1. List household and close contacts.
2. Screen for leprosy and tuberculosis.
3. Offer SDR-PEP to eligible contacts.
4. Educate contacts on early symptoms and stigma reduction.
5. Follow national programme reporting procedures.

E. Reaction / neuritis management

- **Urgent referral or treatment** if new sensory loss, weakness, nerve pain, eye involvement, ulceration or severe systemic symptoms.
- Use corticosteroids for neuritis/new nerve-function impairment.
- Continue MDT unless specialist advice says otherwise.
- Add self-care, splinting, eye care, footwear and rehabilitation.

2.7 Overall Conclusion

As of **29 April 2026**, the WHO **2018** guideline remains the core GRADE-based standard for leprosy diagnosis, treatment and prevention. WHO's current clinical position is stable: diagnose primarily through clinical assessment, classify PB/MB, treat all patients with MDT using **rifampicin, dapsone and clofazimine**, provide **6 months** of treatment for PB and **12 months** for MB disease, and offer **single-dose rifampicin PEP** to eligible contacts where safe screening systems exist [WHO publication page](#), [WHO treatment page](#), [WHO leprosy fact sheet](#).

Post-2018 evidence strengthens implementation confidence for SDR-PEP and contact tracing, confirms the importance of AMR surveillance, and reinforces disability prevention through early reaction and neuritis management. However, major evidence gaps remain in early diagnostics, optimized prophylaxis, drug-resistant treatment, steroid-sparing reaction therapy, and structured participation of affected persons and youth in future leprosy guideline development.